Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 1.0 | **Date:** 04-Apr-2026

---

1. Administrative & Study Identification

Full Study Title A Phase II, Open-Label, Multicenter Study to Evaluate the Optimization of the Cytokine Release Syndrome Profile for Glofitamab in Combination With Gemcitabine Plus Oxaliplatin in Patients With Relapsed/Refractory Diffuse Large B-Cell Lymphoma **Short Title/Acronym** Glofit-GemOx CRS Optimization Study **Protocol Number** GO45434 / 2024-516791-15-00 **NCT Number** NCT06806033 **Posting ID** 865919250625775059 **Trial Status** RECRUITING **Sponsor/Company** Hoffmann-La Roche (Roche) **Investigational Product** Glofitamab in combination with gemcitabine (trade names: Gemzar, Infugem) and oxaliplatin (Glofit-GemOx), with obinutuzumab pre-treatment **Phase of Development** Phase II **Medical Monitor** Hoffmann-La Roche Clinical Operations Lead

---

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the incidence of cytokine release syndrome (CRS) for glofitamab in combination with gemcitabine plus oxaliplatin (Glofit-GemOx) in patients with relapsed/refractory (R/R) diffuse large B-cell lymphoma (DLBCL). **Secondary Objectives:** To evaluate the incidence of serious CRS events, assess CRS frequency relative to the start of glofitamab infusions, and determine the complete response (CR) rate as evaluated by an independent review facility (IRF) and the investigator.

2.2 Background & Rationale Diffuse Large B-Cell Lymphoma (DLBCL) is a fast-growing blood cancer that can be difficult to treat if it relapses or is refractory to standard therapies. The main goal of this trial is to study the frequency and severity of cytokine release syndrome (CRS) in participants with DLBCL who are using a combination of glofitamab + gemcitabine + oxaliplatin (Glofit-GemOx) followed by glofitamab-only treatment. Optimizing the CRS profile is crucial to safely manage patient reactions and expand access to this therapy.

---

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target N: 100 participants **Number of Sites:** Multicenter **Estimated Study Start:** 05-Mar-2025 **Estimated Primary Completion:** 31-Jan-2027

---

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years with histologically confirmed Diffuse Large B-Cell Lymphoma (DLBCL), not otherwise specified (NOS). 2. Relapsed or refractory (R/R) disease, defined as: relapsed = disease that has recurred following a response that lasted ≥ 6 months after completion of the last line of therapy; refractory = disease that did not respond to or that progressed < 6 months after completion of the last line of therapy. 3. At least one line of prior systemic therapy. Participants who have failed only one prior line of therapy must not be a candidate for high-dose chemotherapy followed by autologous stem cell transplant (ASCT). 4. At least one bi-dimensionally measurable (> 1.5 cm) nodal lesion, or one bi-dimensionally measurable (> 1 cm) extranodal lesion, as measured on CT scan. 5. Eastern Cooperative Oncology Group (ECOG) status of 0, 1, or 2. 6. Adequate hematologic and renal function.

4.2 Exclusion Criteria 1. Prior enrollment in Study GO41943, GO41944 (STARGLO), or Study GO44900. 2. Participant has failed only one prior line of therapy and is a candidate for stem cell transplantation. 3. History of transformation of indolent disease to DLBCL. High-grade B-cell lymphoma with MYC and BCL2 and/or BCL6 rearrangements, and high-grade B-cell lymphoma NOS. Primary mediastinal B-cell lymphoma. 4. History of severe allergic or anaphylactic reactions to humanized/murine monoclonal antibodies, or contraindications to obinutuzumab, gemcitabine, oxaliplatin, or tocilizumab. 5. Prior treatment with glofitamab or other bispecific antibodies targeting both CD20 and CD3; or prior treatment with gemcitabine or oxaliplatin. 6. Peripheral neuropathy or paresthesia assessed to be Grade ≥ 2 at enrollment. 7. Treatment with radiotherapy, chemotherapy, immunotherapy, immunosuppressive therapy, or any investigational agent within 2 weeks prior to first study treatment; treatment with monoclonal antibodies within 4 weeks. 8. Current or history of primary/secondary CNS lymphoma, stroke, epilepsy, CNS vasculitis, or neurodegenerative disease. 9. Significant cardiovascular disease or significant pulmonary disease. 10. Known active bacterial, viral, fungal, mycobacterial, parasitic infections (positive for SARS-CoV-2, TB, HBV, HCV, chronic active EBV). 11. Known or suspected history of hemophagocytic lymphohistiocytosis (HLH) or progressive multifocal leukoencephalopathy (PML). 12. Prior solid organ transplantation or prior allogenic stem cell transplant. 13. Active autoimmune disease requiring treatment; prior treatment with systemic immunosuppressants within 4 weeks; ongoing systemic corticosteroid use risking adrenal insufficiency. 14. Pregnancy or breastfeeding, or intention of becoming pregnant. Clinically significant history of cirrhotic liver disease.

---

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Participants will receive glofitamab + gemcitabine + oxaliplatin (Glofit-GemOx) followed by glofitamab-only treatment. Obinutuzumab pre-treatment is administered prior to glofitamab infusions to mitigate CRS risk. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Combination therapy followed by monotherapy across scheduled 21-day cycles.

5.2 Concomitant & Prohibited Therapy Permitted: Tocilizumab is permitted and utilized as indicated for CRS management. **Prohibited:** Live, attenuated vaccines within 4 weeks prior to first dose; systemic immunosuppressive medications (e.g., cyclophosphamide, azathioprine, methotrexate) within 4 weeks prior to first dose; ongoing systemic corticosteroids.

---

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Scans/Labs, Infection Panels
Baseline	Day 0	Pre-treatment administration prior to Glofitamab
Treatment	Cycle based	Safety Labs, Glofit-GemOx infusions, CRS monitoring, Efficacy Scans
End of Study	Variable	Post-treatment follow-up visits, resolution of AEs, long-term survival tracking

---

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. Incidence of cytokine release syndrome (CRS).

7.2 Secondary Outcomes 1. Incidence of serious cytokine release syndrome (CRS) events. 2. CRS frequency relative to the start of glofitamab infusions. 3. Complete response (CR) rate as determined by independent review facility (IRF) and the investigator.

---

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Intensive monitoring for signs of CRS during and after infusions. General monitoring for neuropathy, cytopenias, and infections. **Serious Adverse Events (SAEs):** Standard 24-hour reporting. Pre-emptive protocols are in place for the administration of tocilizumab or additional steroids in the event of severe CRS. **Data Monitoring Committee (DMC):** Managed by Hoffmann-La Roche safety teams.

---

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for efficacy endpoints; Safety Set for AE/SAE evaluations and CRS profiling. **Sample Size Justification:** Target $N = 100$ is powered for a Phase II trial to reliably estimate the incidence of CRS and complete response (CR) rates. **Handling of Missing Data:** Standard multiple imputation or censoring rules applied per oncology guidelines.

---

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** High-frequency onsite visits during the first cycles to capture CRS onset and frequency relative to glofitamab infusions. **Audit Trail:** Standard sponsor eCRF locking, query resolution, and data clarification forms.

---

11. Study Identifiers & Governance

Primary ID: GO45434 **Registry Number:** NCT06806033 **Posting ID:** 865919250625775059 **Sponsor/Lead Organization:** Roche (Hoffmann-La Roche) **Trial Status:** RECRUITING **Study Title:** A Study to Evaluate the Optimization of the Cytokine Release Syndrome Profile for Glofitamab in Combination With Gemcitabine Plus Oxaliplatin in Participants With Relapsed/Refractory Diffuse Large B-Cell Lymphoma **Official Regulatory Title:** A Phase II, Open-Label, Multicenter Study to Evaluate the Optimization of the Cytokine Release Syndrome Profile for Glofitamab in Combination With Gemcitabine Plus Oxaliplatin in Patients With Relapsed/Refractory Diffuse Large B-Cell Lymphoma

---

12. Clinical Framework (Lymphoma Specific)

Disease Area / Primary Condition: Diffuse Large B-Cell Lymphoma (Preferred UMLS Name: Diffuse Large B-Cell Lymphoma) **Diagnosis Threshold:** Histologically confirmed DLBCL (NOS) failing at least 1 prior systemic therapy (R/R disease). **Medical Methodology:** Bispecific Antibody (CD20xCD3) combined with Chemotherapy (GemOx). **Optimization Target:** Optimization of the Cytokine Release Syndrome (CRS) safety profile.

---

13. Study Procedures & Methodology

Management Pathway: Clinical monitoring for Glofit-GemOx administration. **Education Protocol (HCP):** Recognition of CRS and standardized administration of tocilizumab/steroids. **Education**

Protocol (Patient): Education on recognizing early CRS symptoms and immediate reporting protocols. **Quality Audit Mechanism:** Tracking of CRS event timing (specifically frequency relative to start of glofitamab), severity grading, and resolution success.

---

14. Observation & Follow-Up Schedule

Total Duration: Active treatment and follow-up until disease progression or end of study. **Onsite Visit Cadence:** Routine visits aligned with 21-day cycles during the treatment phase. **Remote Monitoring Frequency:** Follow-up post-treatment for well-being and disease status (IRF review). **Checklist Review Interval:** Prior to each cycle infusion.

---

15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- | | **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] | ___ |** ***[DD-MMM-YYYY]*** **| |** ***Lead Statistician | [Name] |*** _____ | [DD-MMM-YYYY] |